

EMERGENCY DEPARTMENT WOMEN AND CHILDREN HOSPITAL KUALA LUMPUR

Neonatal Jaundice Order Set (ONLY for Term Infant <15 days and Premature (<37 weeks) Infant <22 days)

PATIENT DETAILS

Date :	Time:	Age:	Weight:	No. of encounter:
Name :				
Address :				Contact No:
D.O.B :	Sex: M / F	IC/Passport:	MRN:	
Allergy:		Co-morbid:		

INITIAL ASSESSMENT

Date & Time of birth	:	_____
Postnatal age (Days / Hours)	:	_____
Birth Weight (kg)	:	_____
Current Weight (kg)	:	_____
Gestational Age (GA)	:	_____
Percentage of weight loss	:	_____
Baby blood group	:	_____
Maternal blood group	:	_____
Cord TSH Level	:	_____
G6PD Status	:	_____
Latest bilirubin level/Date	:	_____/_____

Make sure! Naked, no diaper
 Be extra cautious if :
 1) < 38 weeks and / or
 2) Weight loss >1% per day at less than
 7 days of life from birth weight

☐ TCB: _____
☐ TSB taken

Risk Factors for Severe Neonatal Jaundice

- ☐ Prematurity
- ☐ Low Birth Weight (< 2.5kg)
- ☐ Jaundice in the first 24 hours of Life
- ☐ Mother with Blood Group O / Rhesus Negative
- ☐ G6PD Deficiency
- ☐ Sepsis
- ☐ Lactation failure in exclusive breastfeeding
- ☐ Cephalhaematoma or bruises
- ☐ Babies of diabetic mother
- ☐ Family history of severe NNJ in siblings

***NOTIFY DOCTOR IMMEDIATELY if signs of Encephalopathy or Sepsis**
***Consider admission if TSB below photo level BUT there are concerns for significant weight loss/sepsis/fever/unwell**

DR Signature/ Time	SN/MA Signature/ Time	Initial Management
		Obtain stat serum blood bilirubin
		Obtain Blood Glucose Reading (If clinically indicated)
		Obtain vital signs
		Use Kramer's Rule (Refer to Table 1) If jaundice at level of umbilicus and below, to consider admission
		Encourage breast or bottle feeding on demand
		Document all interventions and observations in nursing chart

Signs of Acute Bilirubin Encephalopathy

(Ask for help if any signs found)

1. Mental status :

- ☐ Sleepy/lethargy/poor suck/irritable/jittery
- ☐ Semi coma/apnoea/unable to feed/seizures/coma

2. Muscle tone

- ☐ Hypotonia, beginning arching of the neck and trunk
- ☐ Retrocollis/opisthotonus/cycling movement or twitching

3. Cry pattern

- ☐ High pitched/shrill/inconsolable
- ☐ Weak cry/absent

Signs of Sepsis (Ask for help if any signs found)

- ☐ Apnoea
- ☐ Difficulty in feeding
- ☐ Feeding intolerance (vomiting, abdominal distension)
- ☐ Temperature instability
- ☐ Petechiae
- ☐ Hepatosplenomegaly

Kramer's Rule (Visual assessment of neonatal jaundice)

Area of Body	Level	Range of Serum Bilirubin (μmol/L)
Head and Neck	1	68 - 133
Upper trunk (above umbilicus)	2	85-204
Lower trunk and thighs (below umbilicus)	3	136-272
Arms and lower legs	4	187-306
Palms and soles	5	≥ 306

***If jaundice extending to soles of feet, urgent referral to paediatrics is warranted**

ATTENDING DOCTOR
SIGNATURE AND STAMP

ATTENDING SN/MA
SIGNATURE AND STAMP

When bilirubin levels known : plot in chart. Use appropriate graph based of presence or absence of risk factor

Total Bilirubin

Age (hours)

Conjugated/
Unconjugated

Photo level

Severe NNJ Blood Profile

☐ FBC
☐ FBP
☐ Reticulocyte count
☐ Direct Coombs Test
☐ G6PD (if not screened)
☐ Mother Blood Group (if not screened)
☐ Baby Blood Group (if not screened)
☐ Septic workout if infection suspected
☐ GSH

- Plan

☐ Admit

☐ Inform ward

☐ Discharge disposition

☐ TCA nearest health clinic to repeat TSB as per guideline
☐ If TSB >85 (>14 days in term baby and >21 days in premature 35-37weeks), refer health clinic for prolonged jaundice workout.

Ordered by :		DR ORDERS BASED ON APPROPRIATE RISK GRAPH AND CLINICAL ASSESSMENT
DR Signature/Time		
		If result is below phototherapy line If child clinically well, to allow discharge with a TCA at nearest local health clinic for repeat TSB as per guidelines
		If result is above phototherapy line Admission to paediatrics ☐ Relevant bloods taken as per severe NNJ Blood Profile
		If result above exchange transfusion line Manage as per if result is above pototherapy line Inform pediatric team

ATTENDING DOCTOR
SIGNATURE AND STAMP

ATTENDING SN/MA
SIGNATURE AND STAMP